Three Easy Mistakes When You're Planning Ahead for Mom, and the Habit That Helps With Each

What each one looks like, why it is so easy to make, and what to do instead.

Families who are planning ahead tend to trip in the same three places. None of them is a failing. They are what happens when there hasn't yet been a reason to learn this, and each has a simple habit that helps.

1 Guessing what a change means

WHAT IT LOOKS LIKE Mom tells the same story twice at dinner, or the mail has piled up on the hall table. One of you says "she's just getting older." Another quietly decides it is the start of something. Neither of you knows.

WHY IT HAPPENS From the kitchen table there is no way to tell ordinary aging from a sign that more help is needed. MedlinePlus says it's normal to forget things once in a while, and also that some kinds of forgetting may be signs of a more serious problem. Telling the two apart isn't something a family can do by watching. [1]

> **WHAT TO DO INSTEAD** Write down what you saw, with the date, and leave out what you think it means. If Mom is worried about her memory, MedlinePlus's advice is to see a health care provider. Bring the notes along. [1]

2 Assuming you already know what Mom would want

WHAT IT LOOKS LIKE "Mom would never leave that house." "She'd want to move closer to us." Everyone at the table is sure, and nobody has asked her.

WHY IT HAPPENS You do know her well, and it's an awkward thing to bring up when nothing is wrong. The Administration for Community Living notes that most people prefer to remain in their homes for as long as possible. That may be true of Mom. It's still a guess until she says it. [2]

> **WHAT TO DO INSTEAD** Ask her, at a quiet time, in open questions: where she would prefer to receive care if she ever needed more help, how much staying at home matters to her, and who she wants involved in decisions. Write down what she says in her own words, with the date.

3 Treating the kinds of care as one thing, or as a decision for today

WHAT IT LOOKS LIKE "A home" gets used for everything from a helper a few mornings a week to a nursing home. Or the family starts debating assisted living for a mother who still does her own shopping.

WHY IT HAPPENS Nobody explains the levels of care until a family needs them, so the words blur together. In general, long-term care can be provided at home, in the community or in a facility, and for many people a blended approach works best. [3] [2]

> **WHAT TO DO INSTEAD** Learn the words, one plain line each, and write down your questions. Which kind of care would fit Mom, and when, depends on her needs, your state and the providers near her. Leave that to the people who can assess her.

The one habit behind all three

Write down what you saw and what she said, with the date. Ask instead of guessing, and leave what it means to the people who can assess her.

What this guide doesn't do

It doesn't tell you what a change means, what Mom wants, or which kind of care would fit her, and it isn't medical, legal, financial or tax advice. It names three habits of thinking, the same three the Medicaid Planning Roadmap sees at this step.

Ready for the next step?

You know which guesses to set aside. The care interview replaces them with a structured look at Mom's situation, area by area, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

The statements about memory, about where most people prefer to live and about where care is provided come from the sources below. The three mistakes are the Medicaid Planning Roadmap's own.

1. MedlinePlus (National Library of Medicine), Memory | MedlinePlus
2. Administration for Community Living (LongTermCare.gov), Where Can You Receive Care?
3. Administration for Community Living (LongTermCare.gov), What is Long-Term Care?

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